BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Relative stages explain the order of care; they are not equally spaced appointments. full blood count (FBC), kidney and liver profile at baseline and before each treatment; National Cancer Control Programme (NCCP) also explicitly lists Day 8 FBC. Check neuropathy, daily function, infection and previous infusion reactions before every dose. Confirm whether Day 22 is prescribed.

Check	Before starting	Early treatment	Ongoing treatment	Pause / dose change	Completion / next decision
Blood / kidney / liver	Baseline FBC, kidney and liver tests	Before each weekly dose	Repeat before each planned dose	Check recovery before restarting	Follow unresolved abnormalities
Symptoms / premedication	Review symptoms, medicines and reaction history	Check premedication, neuropathy and function	Review before every treatment	Record worst and current symptoms; clinician confirms changes	Review persistent symptoms and recovery
Heart / breathing	Assess history and symptoms	Infusion observations and symptom review	Extra tests if symptoms or risk require	Urgent assessment for chest pain, fainting or new breathlessness	Agree follow-up for unresolved symptoms
Cancer / course	Confirm 00226d small-cell lung cancer (SCLC) and off-label consent	Agree response review and actual appointment plan	Continue while beneficial and tolerated	Reassess benefit, toxicity and priorities	Agree next treatment or supportive-care plan

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00226 v9 · official regimen · HPRA conventional paclitaxel SmPC; 9 January 2026

THE WHOLE TREATMENT JOURNEY

Weekly paclitaxel · relapsed or refractory SCLC: treatment and checks

Lung · relapsed / refractory SCLC

Paclitaxel 80 mg/m² is infused on Days 1, 8, 15 and 22 of a 28-day cycle. The consultant may instead prescribe Days 1, 8 and 15. Each planned weekly dose needs review. Continue while helping and tolerated; NCCP does not set a fixed cycle cap.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Early treatment	Ongoing treatment	Pause / dose change	Completion / next decision
Treatment plan NCCP 00226	Confirm indication and prescription	Days 1/8/15/22 every 28 days	Confirm Day 22 or consultant-selected omission	Clinician review before restart	Agree the next plan

Care / check	Before starting	Early treatment	Ongoing treatment	Pause / dose change	Completion / next decision
Blood / kidney / liver NCCP 00226 · SMPC	Baseline FBC, kidney and liver tests	Before each weekly dose	Repeat before each planned dose	Check recovery before restarting	Follow unresolved abnormalities
Symptoms / premedication NCCP 00226 · SMPC	Review symptoms, medicines and reaction history	Check premedication, neuropathy and function	Review before every treatment	Record worst and current symptoms; clinician confirms changes	Review persistent symptoms and recovery
Heart / breathing NCCP 00226 · SMPC	Assess history and symptoms	Infusion observations and symptom review	Extra tests if symptoms or risk require	Urgent assessment for chest pain, fainting or new breathlessness	Agree follow-up for unresolved symptoms
Cancer / course NCCP 00226 · SMPC	Confirm 00226d SCLC and off-label consent	Agree response review and actual appointment plan	Continue while beneficial and tolerated	Reassess benefit, toxicity and priorities	Agree next treatment or supportive-care plan

What each check is for

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Safety review

Before each weekly treatment

FBC, kidney and liver profile at baseline and before each treatment; NCCP also explicitly lists Day 8 FBC. Check neuropathy, daily function, infection and previous infusion reactions before every dose. Confirm whether Day 22 is prescribed.

Ask / act: The team reviews results, current symptoms and the prescription.

Sources: NCCP 00226 · SMPC

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Benefit and treatment choice

At consultant-agreed response reviews

A small uncontrolled study found some tumour responses to weekly paclitaxel in previously treated SCLC. The study used six weekly doses followed by two weeks without treatment, which differs from this NCCP schedule. It cannot establish a survival advantage over other treatment or supportive care.

Ask / act: Discuss continuing, changing or stopping treatment according to benefit, toxicity and priorities.

Sources: NCCP 00226 · TRIAL

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-29 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.25